



THE PLAN

Start here

Read this one page first. It tells you what everything else is and what to do this week.

Status labels apply throughout: KNOWN, VERIFIED, ASSUMPTION, ESTIMATE, TO VALIDATE, PROPOSED. Nothing in this document claims traction, approvals, partners or revenue that do not exist. Governing file: FACTS_BASE.

PREPARED FOR

The Founder, Vytalix

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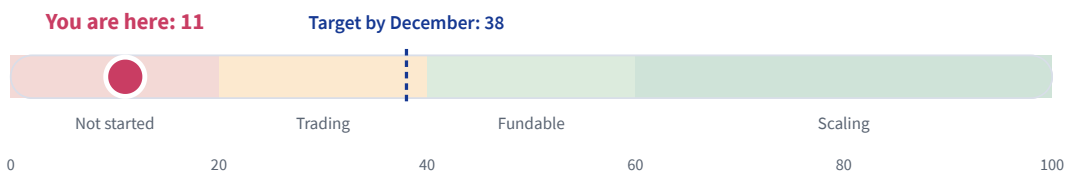
Read this one page first. It tells you what everything else is and what to do this week.

In one minute

- You have a good idea, a name, a logo, a prototype and one past government conversation. You do not yet have a company, a signed agreement over who owns MaternaLink, a customer, or any money coming in.
- The business scores **11 out of 100** today. That is normal for an idea that has not started trading. The plan takes it to about 38 by December.
- The biggest single risk is not competition. It is that nobody has written down who owns MaternaLink.
- The fastest money is advice and consulting, not the product. The product is the long-term value.
- Five things must be fixed before you speak to a single investor. They are listed below and they take about four weeks.

Where the business stands today

A score out of 100. Anything under 20 means the company has not started trading.



Where you are now, and where the 90-day plan takes you.

What to do

Action	Who	By when
Speak to David Agunede and write down who owns MaternaLink	You	Day 3
Check the name is free, then register the company	You, then an accountant	Day 10
Stop sending the 2025 AI slide deck; put the prototype behind a password	You	Day 3
Get your first paying advisory client	You	Day 45
Get one hospital to agree to a trial	You	Day 60

The five blockers, in plain words

1. **No company.** You cannot open a bank account, sign a contract, apply for a grant or take investment without one.
2. **Nobody owns MaternaLink on paper.** David Agunede signs his emails as founder and chief executive of Maternal Link. Until there is a signed agreement, you cannot honestly tell anyone the company owns it.

- 3. **The old slide deck makes medical claims you cannot back up.** It says the software predicts pre-eclampsia, sepsis, bleeding and blood clots. There is no evidence for that, and software that predicts illness is legally a medical device. Withdraw the deck.
- 4. **No revenue.** No client has paid you anything.
- 5. **No hospital, no clinician.** Nobody in the health service has agreed to try it.

None of these is unusual at this stage. All five are fixable in about four weeks. Doing them in this order is what turns a plan into a company.

What you should read, and when

What to read, and when

Seventeen documents. You do not need to read them all at once.

Read this week	Read before selling	Read before investors	Look up when needed
00 Start here	03 Who buys it	05 The money	08 Running the company
01 Where you stand	04 How we make money	10 Partners	09 Legal and safety
02 What we are building	07 Getting customers	13 The investor list	11 Brand
06 The plan	14 Pricing	15 How big this gets	12 Structure and markets
			16 What you protect

Everything else is reference: the spreadsheets, the developer documents and the investor and partner lists.

You do not need to read everything at once. Four documents this week is enough.

Read this week	Why	
00 Start here (this page)	What to do first	
01 Where you stand	The honest audit of what exists today	
02 What we are building	The product, and why the AI idea is parked	
06 The plan	Your first 90 days, day by day	

Read before you sell	Read before you meet investors	Look up when you need it
03 Who buys it	05 The money and the raise	08 Running the company
04 How we make money	10 Partners	09 Legal, data and safety
07 Getting customers	13 How to use the investor list	11 Brand and websites
14 Pricing the full product	15 How big this can get	12 Structure and which countries
	16 What you can protect	

Everything else is reference material: spreadsheets, the documents for your developer, and the investor and partner lists.

What is in the pack

Folder	What is in it
docs/	The seventeen documents above
product/	Four documents to hand to a software developer
assets/	Things you send to other people: company profile, service list, proposal template, one-pager, four slide decks
finance/	The three-year financial model
investors/	316 investors, the top 25, call lists, scripts, and 74 partner organisations
operations/	Weekly numbers to track and pipeline trackers
diagrams/	The charts used in these documents
exports/	Everything again as PDF, Word, Excel and PowerPoint
website/ and maternalink-site/	The two websites

The two websites are live

- Vytalix: <https://elkelvinwilliams.github.io/Vitalyx/>
- MaternaLink, with a working demonstration dashboard:
<https://elkelvinwilliams.github.io/Vitalyx/maternalink/>

Both still contain placeholders for the company number, address and email. Fill those in once the company exists.

One rule for everything here

Nothing in this pack claims a customer, a partner, an approval, a clinician or a pound of revenue that does not exist. Where something is a guess, it says so. Keep it that way. Every investor and every hospital will check, and being the founder who did not exaggerate is worth more than any slide.

Words explained

Word	What it means
Pre-seed	The first proper round of investment, usually £250,000 to £500,000, raised before a product is selling
Pilot	A short trial with one hospital or programme, usually 12 weeks, to see whether the thing works
Due diligence	The checking an investor does before giving you money. They will find anything you exaggerated
ESTIMATE	Our best guess, not a fact
TO VALIDATE	Something you must check before relying on it



THE PLAN

Where you stand

An honest look at what exists today, what is missing, and what to fix first.

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An honest look at what exists today, what is missing, and what to fix first.

In one minute

- Everything that exists today is an idea, a name, a logo, one prototype and one conversation that went nowhere. That is the true starting point.
- The business scores **11 out of 100**. Under 20 means a company that has not started trading.
- Ten problems are listed below. The first four block everything else and take about four weeks to clear.
- Ten opportunities are listed too. The best one is not the product. It is selling your own advice while the product is built.
- Nothing here is a criticism of the idea. The idea is sound. The company around it does not exist yet.

What to do

Action	Who	By when
Agree in writing who owns MaternalLink	You and David Agunede	Day 3
Register the company after checking the name	You	Day 10
Withdraw the 2025 AI deck and lock the prototype	You	Day 3
Book a code and security check of the prototype	A developer you pay	Day 14
Re-score this document	You	14 October

What was actually reviewed

What	Where it was	What it is	Verdict
The code repository	GitHub	One file with one line of text	Empty
Your written brief	This project	A clear, complete description of the company you want to build	The strongest thing you had
"Black Maternal health" deck, October 2025	Google Drive	13 slides proposing AI that predicts pre-eclampsia, sepsis, bleeding and blood clots	An idea only. No data, no model, no clinician, no permission to use patient data
Ekiti State proposal, March 2026	Email	Sent by David Agunede as "Founder and CEO, Maternal Link". £4.5m for 15,000 women, rising to £36m	A proposal with no reply. The price is far above anything comparable
The prototype dashboard	maternal-health-psi.vercel.app	A working website with patients, alerts and a risk engine that suggests clinical actions	Real, but nobody has checked who owns the code or whether it is secure
Kemek Enterprise investor files	Google Drive	Your separate property business, with its own 128-name investor list	Not Vytalix. It shows you can run a disciplined process. It also splits your time
Public record for "Vytalix"	Web search	No company found. Similar names exist in the UK and the US	The name is at risk until checked properly

Nothing else was found. No business plan, no financial model, no website, no brand, no contracts, no customers, no revenue, no grants, no partners, no clinicians, no policies, no insurance.

The score, area by area

Scoring: 0 means nothing exists. 3 means an idea or a draft. 5 means a usable draft that nobody has tested. 7 means it works and there is proof. 10 means best in class.

Area	Score	Why
Company and governance	0	No registered company, no board, no shareholder agreement
Name and brand	1	A logo exists. The name has not been checked and clashes with others
Business model and prices	1	One price existed and it was wrong
Advisory and consulting	1	An idea. No offers, no collateral, no clients. The fastest money in the company
MaternaLink	2	A prototype and a concept. No clinician, no evidence, no plan for regulation
Clinical safety	0	No safety officer, no hazard log, no statement of what the product is for
Data protection and regulation	0	Not registered with the ICO. No data protection assessment. No thinking about medical device rules
Who owns the intellectual property	0	Nothing written down with David Agunede or with whoever wrote the code
Technology	1	One prototype of unknown quality. No domain, no email, no cloud account
E-learning	0	Nothing exists
Proof that customers want it	1	One meeting in Nigeria. No UK conversations at all
Knowing the competition	1	The Ekiti proposal did not mention that the state already has a funded programme
Partners	1	One ministry meeting. No agreements
Sales system	0	No contact list, no scripts, no proposals
Marketing	0	No website, no company page, nothing published
Money and accounts	0	No model, no budget, no bank account, no accountant
Ready for investors	1	You have shown you can run a list. Vytalix has no deck, no data room, no tax clearance
Team	1	One founder and one informal collaborator with unclear roles
Routines and reporting	0	None
Credibility: advisers, evidence, press	0	None
Total	11 / 100	Pre-formation

What you have, what you are building, what comes next

Say it in this order to investors. Never inflate the first column.

What you have

- A full company plan
- A prototype (needs an audit)
- A past government conversation
- A logo and two websites

Building now

- The company itself
- Ownership agreement
- First advisory clients
- A hospital trial

Next

- First paying customer
- A signed pilot
- Advisers on board
- Pre-seed round

Say it in this order to anyone who asks. Never inflate the first column.

What is strong and what is weak

Strong. A precise founder brief. A genuinely important problem with real political attention behind it in the UK. A live conversation in Nigeria and a working prototype, which most founders at this stage do not have. Proof that you can run an investor list properly.

Weak. No company, no ownership, no brand protection. Medical claims with nothing behind them. A price built on nothing. One founder doing everything, across two businesses. No web presence at all.

The chances. Selling your own advice pays within weeks. A re-scoped MaternaLink can be trialled without becoming a medical device. Grants suit this story. Women's health and impact investors are actively looking for something like it. Partnering with the Nigerian incumbent turns a threat into a route in.

The threats. A name clash with two US health-tech companies. Regulatory trouble if any prediction feature ships. A falling-out over who owns MaternaLink. Slow, political government sales in Nigeria. An investor finding all of this in ten minutes.

The ten problems, worst first

#	Problem	Why it matters	What fixes it	By
1	Nobody owns MaternaLink on paper	Blocks funding, contracts and trials. Risks a dispute	Meet this week, agree heads of terms, then a solicitor drafts a deed	Day 7
2	No company	Cannot contract, bank, apply for grants or take investment	Check the name, then register	Day 10
3	Medical claims with no evidence	Regulatory and reputational risk. Fails any investor check	Withdraw the deck. Re-scope the product. Rewrite what it does	Day 3
4	The name may not be yours	Two US health-tech companies use near-identical names	Trademark searches, then file	Day 20
5	No money coming in	Your time is the only funding	Launch three advisory offers, hold 40 conversations	Day 45
6	The £300 per woman price	Anyone who knows the market will see it is wrong	Replace with the new price book	Day 21

#	Problem	Why it matters	What fixes it	By
7	No clinician anywhere near this	You cannot build maternity software without midwives and doctors	Recruit four advisers	Day 60
8	The prototype is unchecked	May contain insecure code, other people's work, or real patient data	Pay for a code and security audit	Day 14
9	Nobody in the UK has been asked	Your home market is untested	15 interviews with midwives and maternity leads	Day 60
10	Two businesses, one of you	Everything waits for you	Write down your time split. Get help with admin	Day 14

The ten opportunities, best first

#	Opportunity	Why it is good	First step
1	Sell your own advice	Cash in weeks, references, credibility	Publish three offers, start 40 conversations
2	MaternaLink as a care-coordination service	The product story, without the medical device problem	Write what it is for, get one clinician to agree
3	Work with the Nigerian incumbent, not against	Keeps the government relationship alive	Reply to Ekiti offering to complement, not replace
4	Grants	£50,000 to £500,000 without giving away shares	Check the Innovate UK and SBRI Healthcare calendars
5	Women's health and impact investors	Better terms and aligned expectations	Work the A-list after the blockers are cleared
6	Publish one research report	Authority, press, inbound interest	Outline it in week five
7	Training for midwives and health workers	Repeat income, low regulatory risk	Two pilot modules with an NGO
8	Consulting for companies entering the NHS or Africa	High margin, repeatable	Package two fixed-price sprints
9	A university research partnership	Turns the 2025 AI idea into fundable research	Approach a maternal health research group
10	Telecoms and insurers in Nigeria	Reach without government budgets	Keep on the partner list for year two

What creates value, and what creates risk

Brings money in now	Builds credibility	Makes investors comfortable	Creates real risk
Advisory days, consulting projects, grant-funded work	Advisers, a published report, a real trial, a professional website	A registered company, clean ownership, sensible prices, a named pipeline	Prediction claims, health data without safeguards, using the prototype with real patients, unclear purpose

Words explained

Word	What it means
Heads of terms	A short written summary of what two people have agreed, before the lawyers write the full contract
ICO	The Information Commissioner's Office, the UK regulator you must register with to handle personal data
Data protection impact assessment	A written check of what could go wrong with people's data, required before you handle health information
Hazard log	A list a health software team keeps of everything that could harm a patient, and what stops it
Medical device	Anything, including software, whose purpose is to diagnose, predict or treat. Tightly regulated



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What we are building

This document says what MaternaLink is, what it is not, and what gets built first.

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This document says what MaternaLink is, what it is not, and what gets built first.

In one minute

- MaternaLink keeps a pregnant woman connected to a real midwife between appointments, and for a year after the birth.
- The old idea was an AI "risk score". It is parked. It is unproven and it would be a medical device.
- The first product is home blood-pressure monitoring for high-risk pregnancy, at one hospital.
- The software never diagnoses. It moves a reading to a named human, with a deadline to act.
- Everything here is PROPOSED (a plan, not a fact). MaternaLink is in development and is approved by nobody.

What to do

Action	Who	By when
Stop sending the 2025 AI deck to anyone; put the prototype behind a login	You	Day 3
Write the one-page intended-use statement and get a clinician to sign it	You	Day 14
Ask your regional Health Innovation Network (the NHS body that introduces new products to hospitals) which two hospitals want home blood-pressure monitoring	You	Week 2
Interview 10 midwives and obstetricians about what happens when blood pressure rises between appointments	You	Week 4
Get one hospital to sign a letter of intent for a 12-week evaluation	You	Month 2

The problem, in plain words

A pregnancy lasts about 280 days. A woman spends perhaps 10 to 15 hours of that in front of a clinician. Almost all of pregnancy happens where no clinician can see.

That is where women get into trouble. Blood pressure rises. Symptoms start. Nobody notices in time. The national reviews keep finding the same thing: women are not listened to, and escalation fails when it matters. Ockenden (2022) reviewed around 1,500 families' cases. Kirkup (2022) reviewed 202 cases in East Kent.

Black and Asian women are failed most. MBRRACE-UK is the UK national report into why mothers die. It is run by Oxford's National Perinatal Epidemiology Unit. It reports Black women at roughly 2.8 times the risk of White women. It reports Asian women at roughly 1.7 times. **Verify the current figure and report year before using it.** Never say "3x" without a source.

Care also stops too early. Midwives hand over at 10 to 28 days. Most late maternal deaths happen after that, in the year following birth, and mental-health causes lead them.

Why the old AI idea does not work yet

The 2025 deck proposed one AI score, the "Maternal Instability Score", predicting pre-eclampsia, sepsis, haemorrhage and blood clots. Six honest reasons it fails today.

#	Reason	What it means for you
1	The NHS already has an official early warning score	NHS England published a national Maternity Early Warning Score with the RCOG and RCM in 2023 to 2024. Midwives are told to use it. A rival score is a liability, not an advance.
2	No evidence that voice predicts these conditions	Voice has some published evidence in mental-health screening. It has none for predicting these physical obstetric emergencies.
3	One score for four conditions makes no clinical sense	Pre-eclampsia, haemorrhage, sepsis and clots differ in cause, timing and treatment. Clinicians will reject a single number that mixes them.
4	Rare events need tens of thousands of pregnancies	Each of these events is rare per pregnancy. A small pilot cohort cannot train or check such a model.
5	Ethnicity as a prediction input is being removed from medical tools	It has already been taken out of other calculators. Ethnicity belongs in checking who you are failing, not in the prediction itself.
6	It would be a medical device	Software that flags risk to inform care is regulated in the UK. ESTIMATE (our best guess, not a fact): 2 to 3 years and over £1m before a single sale.

And the finding that changes the design. The Oxford BUMP trials were published in JAMA in 2022. They found that home blood-pressure monitoring on its own did **not** speed up detection of high blood pressure. It did not improve control either, against usual care. The lesson is not that home monitoring fails. The lesson is that the reading is worth nothing on its own. The value sits in what happens after it: who is told, how fast, and what they do.

So the AI score is not deleted. It moves to the back of the plan, behind a research group of consented women, ethics approval and validation. It is the last thing built, not the first.

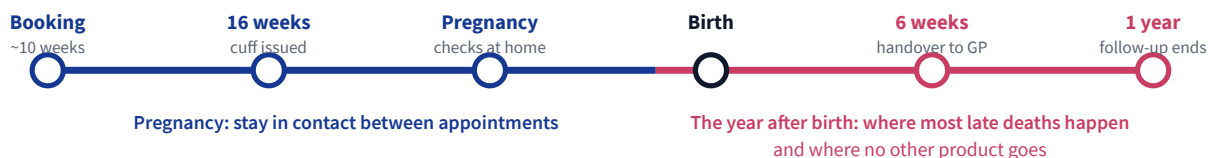
What we build instead

We build the in-between service: the part of care that happens when no clinician is in the room. Seven things.

#	The thing	What it means
1	A record she keeps	It follows her between hospitals, community, private care, and between countries. Consent is set by her, in tiers.
2	Contact that never goes silent	Check-ins by app, SMS or voice note. It works offline, on any phone, in her language.
3	Home readings when her risk warrants it	A validated arm cuff, a urine dipstick read by a phone camera, plus symptoms and medicines.
4	Rules the hospital owns	Thresholds set by the hospital's own clinicians, lined up with the national MEWS and NICE NG133. We ship the rule editor, not the rules.
5	A queue with a deadline	A named midwife is told, with a response time. Every reading, contact and outcome is logged.
6	The year after birth	Follow-up to 12 months: mood, blood pressure, bleeding, infection, feeding, and a proper handover to the GP.
7	Who care is failing	Anonymous reporting of who is reached, escalated, seen and missed, by ethnicity, deprivation and area.

What MaternaLink covers

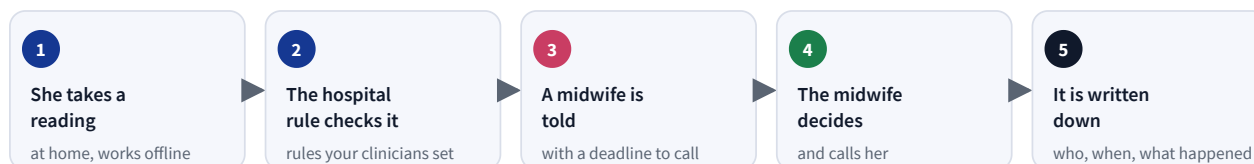
From the first appointment to a year after the birth. The last part is where nobody else goes.



We cover the whole of her care, including the year after birth where no other product goes.

How it works

A person decides, not the software. That is what keeps it safe and legal.



The software never diagnoses, predicts or recommends treatment. It moves information to a human quickly.

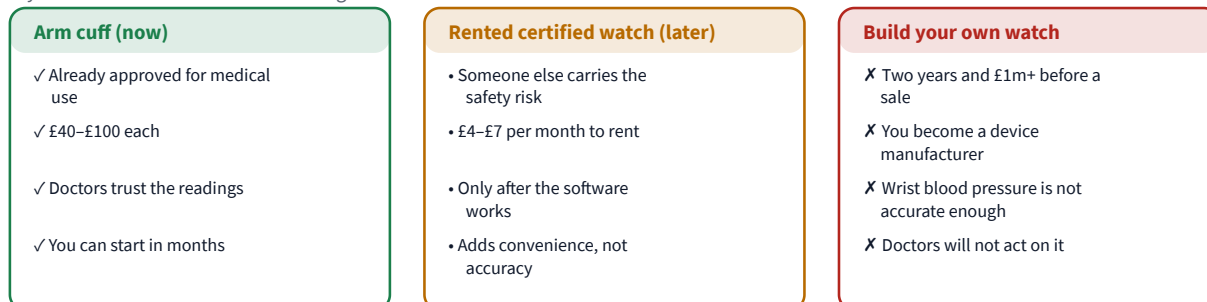
A person decides, never the software. That is what keeps it safe and legal.

The watch

You asked about a maternity watch, issued at week 16 and returned after discharge. Here is the order to do it in.

Cuff now, watch later

Why the wrist device is not the first thing to build.



Start with the cuff someone else already got certified.

- **Arm cuff, now.** It is already approved for medical use. It costs about £40 to £100. Clinicians trust the readings and will act on them. You can start in months.

- **Rented certified watch, later.** Rent a device someone else has certified, so they carry the safety risk. ESTIMATE: £4 to £7 per month. Only after the software works. It adds convenience, not accuracy.
- **Never build your own.** You would become a device manufacturer. That is two years and over £1m before a sale. Wrist blood pressure is not accurate enough, and doctors will not act on it.

This is a fixed decision in `FACTS_BASE.md` (A11): a certified third-party wearable only, never a Vytalix-made device, and only after the v1 software is live.

What it is and is not

It is	It is not
A way to stay in contact between appointments	An app that tells a woman what is wrong with her
A place to put home readings so a human sees them	A machine that scores, predicts or interprets them
Rules written and owned by the hospital's clinicians	Our rules shipped into someone else's ward
A record that follows the woman	The hospital's legal maternity record
A service with a response time and an audit trail	A dashboard with suggested treatments
In development, pre-seed, no customers yet	Approved, validated or assessed by anyone

The five best ideas

Idea	What it does	Why nobody owns it	When
A. Maternity virtual ward, high blood pressure first	Home cuff, phone dipstick, symptom check-ins, hospital-set thresholds, an escalation queue with a response time	General virtual-ward suppliers have no maternity workflow. Maternity record suppliers have no home monitoring. Babyscripts is US-focused.	First. This is the MVP
B. The 365-day companion after birth	Check-ins on mood, blood pressure, bleeding, infection, pain, feeding and contraception, with a handover to the GP	The money sits in three separate budgets: maternity, GP and mental health. So no single team buys it.	v2
C. Woman-held record with graduated consent	A portable record she controls, readable anywhere with her permission, standing alone where no system exists	Making systems talk to each other is slow standards work. Existing suppliers build for institutions, not for women.	v1 into v2
D. Equity reporting for commissioners and ministries	Monthly anonymous figures on who care reaches and misses, instead of a report every few years	It needs continuous data across settings. Only a continuity layer holds that.	v2
E. A consented research group of women	A long-term, diverse group of pregnancies, UK and Nigeria, that universities and industry pay to study under strict rules	It takes years and trust to build. It is not a feature anyone can ship.	v3. The AI score lives here

How it grows

Version	What ships	What must be true before it starts
v1 (now to about 12 months)	Enrolment and consent; check-ins; education signed off by clinicians; secure messaging; home blood-pressure entry; hospital-set thresholds; escalation queue with a response time; handover summary; audit log; equity report; English plus one local language	The MaternaLink ownership agreement with David Agunede is signed. The intended-use statement is signed by a clinician. A DPIA and hazard log exist. A clinical safety officer is appointed. One hospital has signed a letter of intent.
v2 (about months 7 to 15)	The postpartum year; links to hospital systems (BadgerNet, K2, DHIS2); WhatsApp and USSD; partner access with safeguarding controls; blood-glucose diary; triage intake; mental-health questionnaires	A written MHRA classification opinion covers triage, glucose thresholds and questionnaire scoring. v1 has been evaluated at the first site with results written up.
v3 (month 15 and beyond)	The consented research group, and only then a regulated prediction model	Ethics approval, a data-sharing agreement with a hospital or university, retrospective then prospective validation, and an ISO 13485 quality system. ESTIMATE: 2 to 3 years and over £1m.

Words explained

Word	What it means
MEWS (Maternity Early Warning Score)	The official NHS chart that flags a pregnant woman whose vital signs are getting worse. Midwives are already required to use it.
Medical device	Anything, including software, whose purpose is to diagnose, predict or guide treatment. It is regulated by law and cannot be sold without approval.
MHRA	The UK government body that regulates medicines and medical devices.
Virtual ward	Patients looked after at home with monitoring, instead of in a hospital bed. The NHS funds this as a service.
MBRRACE-UK	The UK's national investigation into why mothers and babies die, run from Oxford.
NICE NG133	The national guideline on high blood pressure in pregnancy. Our thresholds must line up with it.
DTAC	The NHS checklist a digital product must pass before a hospital can buy it.
DPIA	A written check of the privacy risks in what you are building. It is required by law before you handle health data.
Hazard log	A list of every way the software could harm someone, and what you did about each one.
Clinical safety officer	A registered clinician who signs off that the software is safe to use. You must appoint one.
Letter of intent (LOI)	A signed letter saying a hospital intends to work with you. It is not yet a contract.
Pre-eclampsia	A pregnancy condition involving high blood pressure. It can become dangerous quickly.
PROPOSED / ESTIMATE / TO VALIDATE	A plan, not a fact / our best guess, not a fact / must be checked before you use it anywhere.



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How we make money

What Vytalix sells, what it charges, and where the first £1,000,000 comes from.

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What to do

The four parts of the business

The ways money comes in

What to charge for advice and consulting

What to charge for MaternaLink

Rules you do not break

The first £1,000,000

Words explained

What Vytalix sells, what it charges, and where the first £1,000,000 comes from.

In one minute

- Advice and consulting bring in cash now. They pay the bills while the product is built.
- MaternaLink is in development. It is not for sale and earns nothing yet.
- Every price here is an ESTIMATE (our best guess, not a fact). None has ever been charged.
- The old £300 per woman per year figure is withdrawn. The new UK price is £18–£36.
- Five sources add up to the first £1,000,000, and services are the biggest of them.

What to do

Action	Who	By when
Put the "from" prices on the website and into the proposal template	Founder	Day 21
Stop using the £300 per woman figure anywhere, in writing or in person	Founder	Now
Test three price points across the first six proposals and log every objection	Founder	Day 45
Ask three UK maternity digital leads what they think of £18–£36 per woman	Founder	Day 31–60
Get a written regulatory opinion before quoting Triage, Glucose or the Watch	Founder	Before any proposal

The four parts of the business

The four parts of Vytalix

Services pay the bills today. The product is the long-term value.

Advisory

Senior advice for health organisations and founders

Earns money now

Consulting

Fixed-price projects: NHS readiness, Africa entry

Earns money now

Health Solutions

MaternaLink, the maternity product

In development

E-Learning

Training for midwives and health workers

Year two

money from services funds the build

Advisory and Consulting earn money today; the maternity product is the long-term value.

The ways money comes in

Fifteen ways, in the order they can start. "Margin" is what is left after the direct cost of doing the work.

What we sell	Who buys it	What we charge	Margin	When it starts	Label
Consulting projects	NHS trusts, private providers, health-tech firms, NGOs	£8k–£45k a sprint; founder day £1,200–£1,800	55–70%	Now	ESTIMATE
Advisory retainers	Health-tech founders, small providers, NGOs	£2,500–£6,000 a month	70–85%	Now	ESTIMATE
Grants	Innovate UK, NIHR, SBRI Healthcare, foundations	£25k to £500k–£1m; year-one target £50k–£150k	Cost recovery only	Prepare now, first award at 6 months	ESTIMATE
E-learning courses	Midwives, nurses, doctors, students	£49–£299 in the UK; £10–£40 regional	85–90%	6 months	ESTIMATE
Corporate training	Trusts, providers, NGOs, scale-ups	£2,500–£6,000 a workshop; £15k–£40k a cohort	60–75%	6 months	ESTIMATE
Shared-revenue partnerships	Health Innovation Networks, consultancies, NGOs	10–30% share, or sub-contract days; £10k–£100k a programme	40–60%	6 months	ESTIMATE
UK public contracts	NHS England bodies, ICBs, councils	£25k–£150k a contract	45–60%	12 months	ESTIMATE
Enterprise contracts	Private hospital groups, pharma teams, insurers	£50k–£300k a year	50–65%	12 months	ESTIMATE
Learning subscriptions	Individual professionals, small clinics	£12–£29 a month; £490–£990 a year for five seats	80–90%	12 months	ESTIMATE
African government work	State ministries, federal agencies, donors	Advice £15k–£80k; programme £100k–£1m+ a year	30–50%	Advice 12 months, programme 24 months	TO VALIDATE
MaternaLink set-up fees	Pilot sites	£15k–£60k a UK site; £25k–£150k a state programme	35–50%	24 months	TO VALIDATE
MaternaLink subscription	Trusts, ICBs, private maternity, African states	UK £30k–£120k a site a year; Africa £5–£40 a woman a year	65–80% once past five sites	24 months	TO VALIDATE
Module licences to other firms	Health-tech vendors, maternity IT firms	£20k–£80k a year each	80% or better	24 months	TO VALIDATE
White-label versions	Insurers, telecoms, hospital groups	£25k–£75k set-up plus £2–£8 a user a year	60–75%	24 months	TO VALIDATE
Technology licensing	Universities, other vendors	Not priceable today	90% or better	24 months or later	TO VALIDATE

What to charge for advice and consulting

Day rates (ESTIMATE). No founder day is ever sold below £1,200.

Role	Day rate	Half day
Founder advisory day	£1,200–£1,800 (list £1,500)	£900
Founder delivery inside a package	£1,500 as the pricing basis	Not sold on its own
Associate consultant	£700–£1,000 (list £900)	£550

Role	Day rate	Half day
Clinical adviser or Clinical Safety Officer	Cost passed on plus 15%	—
Workshop for a day	£4,000 all in (range £2,500–£6,000)	—

Fixed-price packages (ESTIMATE). Price fixed, scope written down, changes agreed in writing. Do not quote C3, C5, C8, C9 or the safety part of C6 until the skill behind them exists and can be shown.

Code	Package	How long	Price
C1	Digital Health Discovery Sprint	2 weeks	£14,000
C2	Business Case and Funding Sprint	3 weeks	£18,000
C3	Digital Maternity Readiness Review	4 weeks	£24,000
C4	UK Market-Entry Playbook	4 weeks	£22,000
C5	Africa Market-Entry and Partner Scan	4 weeks	£20,000
C6	DTAC and Evidence Readiness Sprint	3 weeks	£15,000
C7	Grant and Investor Readiness Sprint	2 weeks	£9,500
C8	Health Inequalities and Maternity Outcomes Review	5 weeks	£28,000
C9	Programme Evaluation Lite	6 weeks	£26,000
C10	Corporate workshop	1 day	£4,000
—	Cohort training programme	4–8 weeks	£15,000–£40,000

Monthly income (ESTIMATE). Never more than five of these at once, because the founder runs out of days.

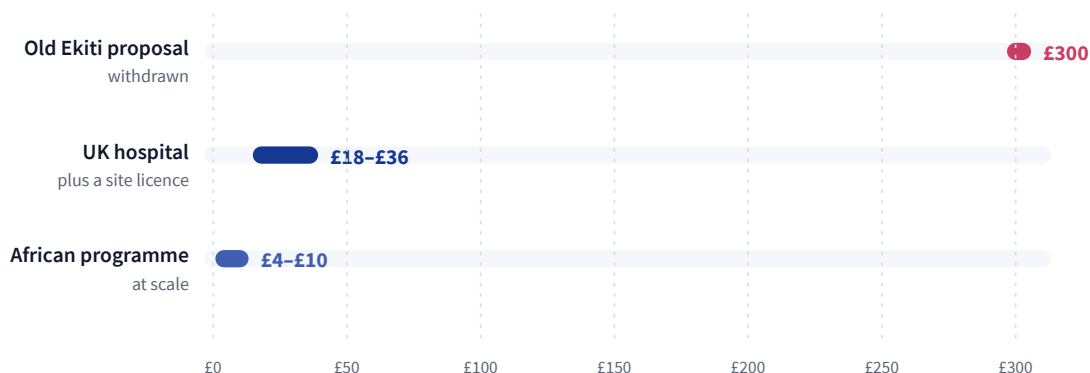
Offer	Days a month	Monthly fee	Minimum term
Retainer — light	1.5	£2,500–£3,000	3 months
Retainer — standard	2	£4,000	3 months
Retainer — extended	3–4	£5,500–£6,000	3 months
Fractional Chief Digital or Strategy Officer	4–6	£5,500–£8,000	6 months

What to charge for MaternaLink

MaternaLink is in development. Nothing is sold until four things are done: ownership documented, the prototype audited, a Clinical Safety Officer and hazard log in place, and a data protection assessment written.

What to charge, per woman per year

The old proposal was far above anything comparable. These are the new numbers.



The withdrawn figure was 8 to 30 times the new prices; the new ones can be defended.

There are two price books because what a UK hospital can pay and what an African programme can pay are ten times apart. Software is priced separately from the watch, always.

Price book A — United Kingdom (ESTIMATE)

What	How it is charged	Price
Core platform	Site licence plus a charge per woman enrolled	£12,000–£30,000 a year, plus £18–£36 a woman a year
Triage module	Per site a year	£15,000–£35,000
Engagement module	Per woman a year	£8–£14
Diabetes tracking	Per woman with the condition a year	£20–£30
Voice diaries	Per woman a year	£3–£5
The watch	Lease a month, or buy	£4–£7 a month, or £120–£220 plus £30 a year
Record integration	One-off, then yearly	£15,000–£40,000, then £4,000–£8,000
Programme analytics	Per site a year	£6,000–£12,000
Training	Per group of up to 25	£2,500–£4,500
Setting it up	One-off per site	£25,000–£75,000
Pilot: 12 weeks, one site, up to 300 women	Fixed	£40,000–£90,000
Yearly service plan	Share of the licence value	18–22% Standard, 25–28% Enhanced, 30–35% Premium

Price book B — African and donor programmes (ESTIMATE)

What	How it is charged	Price
Core platform	Per woman a year, cheaper in bulk	£10 up to 5,000; £7 to 25,000; £5 to 100,000; £4 above

What	How it is charged	Price
Engagement by SMS or WhatsApp	Per woman a year	£3–£5
Triage module	Per facility a year	£1,500–£4,000
Diabetes tracking	Per woman with the condition	£6–£10
Voice diaries	Per woman	£1–£2
The watch	Lease a month, or buy	£2–£4 a month, or £90–£160 plus £15 a year
Health-system integration	One-off, then yearly	£8,000–£25,000, then £3,000
Training	Per group of 30	£1,200–£2,500
Setting it up	One-off per state or programme	£40,000–£120,000
Yearly service plan	Share of the software value	20% Standard, 28% Enhanced, 35% Premium, minimum £12,000

Worked example: a UK trust with 5,000 births a year

Line	Amount
Core licence £20,000 plus 5,000 women at £22	£130,000
Engagement, 5,000 women at £10	£50,000
Triage	£25,000
Analytics	£8,000
Enhanced service, 26% of £213k	£55,400
Setting it up, one-off in year one	£45,000
Year-one total	£313,400
Year-two run-rate	£268,400

Worked example: a state programme covering 15,000 women

Line	Amount
Core, 15,000 women at £7	£105,000
Engagement, 15,000 women at £4	£60,000
Triage, 20 facilities at £2,500	£50,000
Setting it up, one-off	£80,000
Training, 10 groups	£18,000
Enhanced service, 28% of £215k software	£60,200
Year-one total	£373,200, about £25 a woman
Year-two run-rate	About £275,000, about £18 a woman

The withdrawn proposal asked £4.5m for that same group of women. This asks £373,200.

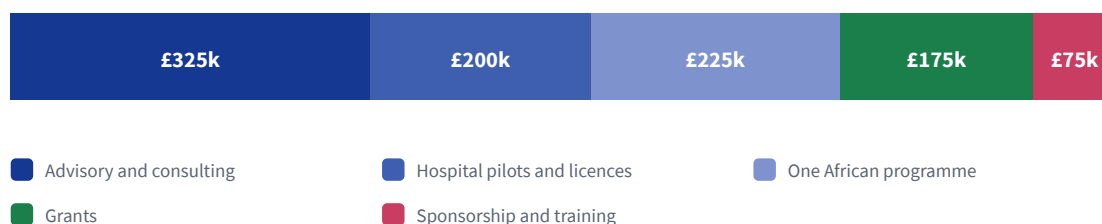
Rules you do not break

- **Pilot first, always.** Sell a fixed-price pilot with a written evaluation. Credit half the pilot fee against year one.
- **Minimum contract size.** £40,000 in the UK, £25,000 for a programme. Below that, walk away.
- **Never discount below the floor.** The floor is £14 a woman in the UK and £4 in a programme, because that is what it costs to serve her. No founder day below £1,200. Bulk discounts only through the published tiers.
- **Hardware is always a separate line.** Never fold the cost of a watch into the per-woman software price.
- Three years buys 5% off, and yearly price rises are capped at inflation plus 2%. Never trade a discount for a testimonial or a logo, and never offer a free MaternaLink pilot outside the agreed design-partner model.

The first £1,000,000

Where your first £1,000,000 comes from

Services first. The product later. Grants alongside.



Mid-points of the ranges. Roughly the first 12 to 18 months.

Services come first, pilots second, one programme third, grants alongside.

Where it comes from	Range	Mid-point
Advisory and consulting	£250k–£400k	£325k
Hospital pilots and first licences	£150k–£250k	£200k
One African programme or NGO contract	£150k–£300k	£225k
Grants	£100k–£250k	£175k
Sponsorship and training	£50k–£100k	£75k
Total at the mid-points		£1,000,000

Roughly the first 12 to 18 months. All ESTIMATE. The order matters more than the total: sell services first.

Words explained

Word	What it means
ESTIMATE	Our best guess, not a fact
TO VALIDATE	We do not know yet and must go and check
Retainer	A client pays the same fee every month for an agreed number of days

Word	What it means
Site licence	A yearly fee a hospital pays for the software, before any per-woman charge
Run-rate	What a year costs once the one-off set-up is behind you
DTAC	The NHS checklist a digital product must pass before a hospital can buy it



THE PLAN

The money: what it costs, what you raise

What the three-year plan says, how much cash it needs, and how you raise it without being caught out.

Status labels apply throughout: KNOWN, VERIFIED, ASSUMPTION, ESTIMATE, TO VALIDATE, PROPOSED. Nothing in this document claims traction, approvals, partners or revenue that do not exist. Governing file: FACTS_BASE.

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The Founder, Vytalix

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Words explained

What the three-year plan says, how much cash it needs, and how you raise it without being caught out.

In one minute

- Three versions of the next three years exist. None has happened. Nothing has been sold.
- The careful version loses money slowly. The middle version spends about £1m building the product.
- Retainers are the best money in the company. Your day rate barely changes anything.
- You raise in four steps. Each step needs proof, not time passing.
- You are not ready to meet an investor today. Fifteen things must be fixed first.

What to do

Action	Who	By when
Take the 2025 predictive claims off every public page and date-stamp the removal	Founder	Day 3
Clear the name, incorporate, open the bank account, appoint the accountant	Founder	Day 12
Sign the MaternaLink ownership deed with David Agunede	Founder and solicitor	Day 30 (hard stop Day 60)
Apply for SEIS/EIS advance assurance	Accountant	Month 1–2
Take no investor meeting until items 1 to 4 below are done and two of 5 to 7	Founder	Day 61

Three versions of the future

Money coming in, three versions of the future

These are scenarios, not forecasts. Nothing has been sold yet.



These are scenarios, not forecasts; the gap between them is mostly the product, not the services.

EBITDA below means profit before interest, tax and the cost of equipment. It is the plain measure of whether the trading is paying for itself.

	Careful	Middle	Ambitious
Three-year revenue	£412,514	£1,474,289	£5,249,714
Three-year EBITDA	–£239,543	–£1,032,201	£113,985
Peak cash needed before any funding	£228,872	£1,058,035	£144,705
Break-even that lasts	Not reached in 36 months	Not reached in 36 months	Month 31
Equity raised in the model	£250,000	£2,400,000	£3,500,000
People employed at month 36	2	8	10

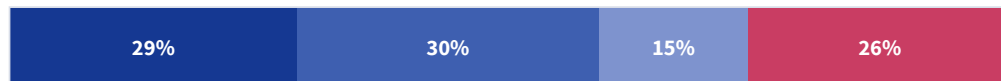
The middle version loses £1m on purpose. That money buys a product, evidence and a team of eight.

Where the money comes from

Where the money comes from (middle case)

Services are most of the money for the first two years.

Total three-year revenue £1.47m



■ Advisory	£424k over three years
■ Consulting	£443k over three years
■ E-learning	£220k over three years
■ MaternaLink	£387k over three years

Services are most of the money for the first two years; the product only matters in year three.

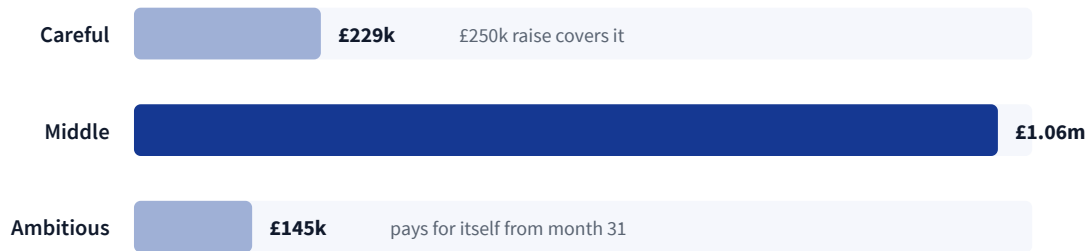
Source	Three-year revenue	Share
Advisory	£423,748	29%
Consulting	£443,333	30%
E-learning	£220,208	15%
MaternaLink	£387,000	26%
Total	£1,474,289	

MaternaLink earns nothing in year one. In year three it is £282,000, and still only 34% of that year.

How much cash you need

How much cash you need before money comes in

The deepest point of the hole, before any investment.



Read it this way: in the middle case the company deliberately spends on building MaternaLink.

The deepest point of the hole, counted with no investment and no grants at all.

	Careful	Middle	Ambitious
Cash needed by month 12	About £4k	About £36k	About £5k
Cash needed by month 24	About £86k	About £253k	About £64k
Cash needed by month 36	£228,872	£1,058,035	£144,705 (peak at month 31)
Lowest the bank balance ever gets	£4,726 in month 5	£6,099 in month 3	£12,750 in month 2

The first 90 days are the tightest point in all three. The rule that follows: never hire the product lead before the money lands.

What actually moves the numbers

What you change	From, to	Three-year revenue	Three-year EBITDA
Retainers held in years two and three	1 to 3	–£94k / +£96k	–£91k / +£93k
Average size of a consulting project	£15k to £30k	–£111k / +£222k	–£72k / +£144k
MaternaLink pilot value a year	£40k to £150k	–£105k / +£225k	–£58k / +£124k
African price per woman per year	£3 to £12	–£61k / +£122k	–£61k / +£122k
Founder day rate	£1,200 to £1,800	–£5k / +£5k	–£5k / +£5k

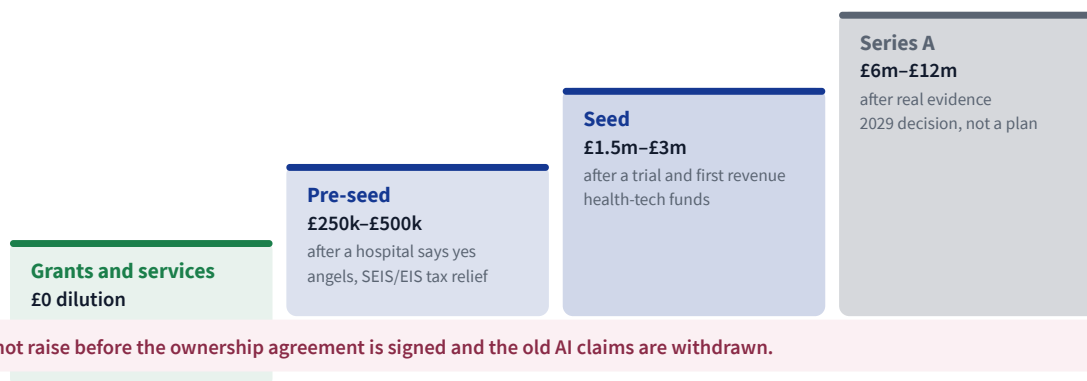
Three things to take from that table.

- **One retainer held for two years is worth about £91,000.** That is more than any single MaternaLink assumption inside three years. Sell retainers first.
- **Bigger projects beat more projects.** Your billable days are already spoken for, so the day rate is almost irrelevant. Capacity is the lever, not price.
- **When MaternaLink goes live matters more the faster you grow.** A 12-month slip costs the middle case £132k of revenue, but it costs the ambitious case £775k of EBITDA and £642k of cash. So do not build the MaternaLink cost base until go-live has a contract date.

Raising money in stages

How to raise money, in stages

Each step is unlocked by proof, not by time passing.



Each step is opened by proof, not by time passing.

Step	Amount	Who puts it in	What must be true first
Grants and services	No shares given up	Innovate UK, SBRI Healthcare, first paying clients	A registered company and a clinical partner letter
Pre-seed	£250k–£500k	SEIS/EIS angels, angel syndicates	Company incorporated; ownership deed executed; 2025 claims retired; advance assurance submitted; one signed services engagement; two named advisers
Seed	£1.5m–£3m	Health-tech and impact funds	Product built, security-tested and in a registered evaluation; one paid pilot signed; safety pack complete; team of three; services at £15k a month with two retainers; clean share register
Series A	£6m–£12m	Growth and impact funds, a strategic as a minority	Recurring MaternaLink revenue of £0.4m–£0.9m, or three UK sites plus one state programme; multi-site evaluation results; team of about ten

Do not open any raise before the ownership agreement is signed and the old AI claims are withdrawn.

SEIS and EIS in plain words. SEIS is a tax scheme that gives people money back when they invest in a small British company. An investor gets 50% of what they put in taken off their income tax. EIS is the same idea for a slightly bigger company, at 30%. This is why UK angels will not invest without it. Never say "SEIS-approved"; say "advance assurance applied for" once that is true.

Limit	Commonly cited figure	Label
SEIS, per company, for its lifetime	£250,000	TO VALIDATE
SEIS, per investor, per tax year	£200,000	TO VALIDATE
SEIS company conditions	Trading under 3 years; assets under £350,000; fewer than 25 full-time staff	TO VALIDATE
EIS, per 12 months	£5,000,000	TO VALIDATE
EIS, lifetime	£12,000,000	TO VALIDATE

TO VALIDATE means we do not know yet. Check every line with an accountant and against gov.uk before saying it to an investor. A £400k pre-seed cannot be all SEIS: it splits into £250k SEIS then £150k EIS.

Are you ready?

Are you ready to talk to investors?

Five of these must be green before your first meeting. Today, none are.

- | | |
|---|--|
| ☐ Company registered | ☒ Financial model with real numbers |
| ☐ Ownership of MaternaLink agreed in writing | ☐ Data room built |
| ☐ Old AI claims withdrawn | ☒ Investor list ready |
| ☐ Name and trademark checked | ☐ Insurance and data registration |
| ☐ First paying client | ☐ Your own time commitment written down |
| ☐ A hospital agrees to a trial | ☐ 15 interviews done |
| ☐ Two advisers signed up | ☐ One grant applied for |
| ☐ Product audited and documented | |

The first four items gate everything else, and none of them is done today.

Done	What must be true	Owner	By when
☐	Company registered, bank account open, accountant appointed	Founder	Day 10
☐	MaternaLink owned or licensed in writing	Founder and solicitor	Day 30
☐	No unproven medical claims anywhere public	Founder	Day 3–14
☐	Name and trademark checked and filed	Founder and trademark attorney	Day 5, filed Day 20
☐	First revenue: one signed piece of work	Founder	Day 45
☐	One hospital or NGO agrees in writing to a trial	Founder	Day 60
☐	Two advisers confirmed, one clinical	Founder	Day 60
☐	Prototype audited, written up, with an intended-use statement	Fractional product lead	Day 30
☐	Financial model updated with real costs, round size chosen	Finance role	Day 60
☐	Data room built	Founder	Day 45
☐	Investor list verified and outreach ready	Founder	Day 55
☐	Data registration, security certification booked, insurance quoted	Founder	Day 30
☐	Your own time commitment written down, the other venture ring-fenced	Founder	Day 7
☐	Fifteen discovery interviews written up, three quotes with consent	Founder	Day 60
☐	One grant application submitted	Founder	Day 75

The rule. Take no first investor meeting until the first four are done and at least two of items 5, 6 and 7. A meeting taken too early costs you that fund for twelve months.

What investors will ask, and the honest answer today

Question	The answer now	The answer at day 90
Do you own MaternaLink?	Not yet in writing	Yes, the deed is signed
Is it a medical device?	Not as re-scoped, but the prototype needs changes	Intended use adopted, prototype changed, a regulatory opinion scoped
Any revenue?	None	The first advisory engagements
Any customer or pilot?	An earlier ministry conversation that went nowhere	One written pilot agreement, fifteen interviews documented
Who is on the team?	A solo founder and one collaborator	Founder, two advisers, a part-time technical lead and a safety officer identified
Why you?	The founder story, still to be written	The founder story plus evidence that you deliver
How much and what for?	£250k–£500k pre-seed	The same, with milestones tied to the model

What kills a raise

What kills it	How an investor finds it	Fixed by
Ownership of MaternaLink not written down	One question: who owns the code. Then the email signature on the old proposal	Day 30, hard stop Day 60
Medical claims nobody has proved	Any copy of the 2025 deck, a web search, or the prototype's own words	Day 3
No company registered	A Companies House search returns nothing	Day 12
The old £300 per woman pricing	The proposal circulates; anyone with African health-tech experience knows the going rate	Day 21 to re-base, Day 60 for the decision

Any one of these ends the conversation on the spot, and some investors will not come back.

Words explained

Word	What it means
EBITDA	Profit before interest, tax and the cost of equipment
Peak cash need	The deepest the bank balance would go with no investment and no grants at all
Pre-seed	The first outside money, usually from individuals rather than funds
Seed	The round that pays for the product and the first proper team
Series A	The round that pays for scaling something already working
SEIS and EIS	Tax schemes that give people money back when they invest in a small British company
Advance assurance	HMRC saying in advance that your company looks eligible for those schemes

Word	What it means
Data room	One organised folder holding everything an investor will ask to see
TO VALIDATE	We do not know yet and must go and check



THE PLAN

The plan: your first 90 days

What you do each week from Monday 14 September to Saturday 12 December 2026, and the four decisions that control it.

Status labels apply throughout: KNOWN, VERIFIED, ASSUMPTION, ESTIMATE, TO VALIDATE, PROPOSED. Nothing in this document claims traction, approvals, partners or revenue that do not exist. Governing file: FACTS_BASE.

PREPARED FOR

The Founder, Vytalix

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8 September 2026

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What you do each week from Monday 14 September to Saturday 12 December 2026, and the four decisions that control it.

In one minute

- Ninety days to turn an idea into a real company that trades and tells the truth.
- Four blocks: fix the basics, build the sales engine, meet investors, launch.
- Four decisions control everything: the company, the ownership, MaternaLink, Ekiti.
- Money comes first. One paying client by 13 October, three by 12 December.
- You speak to no investor until the first three decisions are closed.

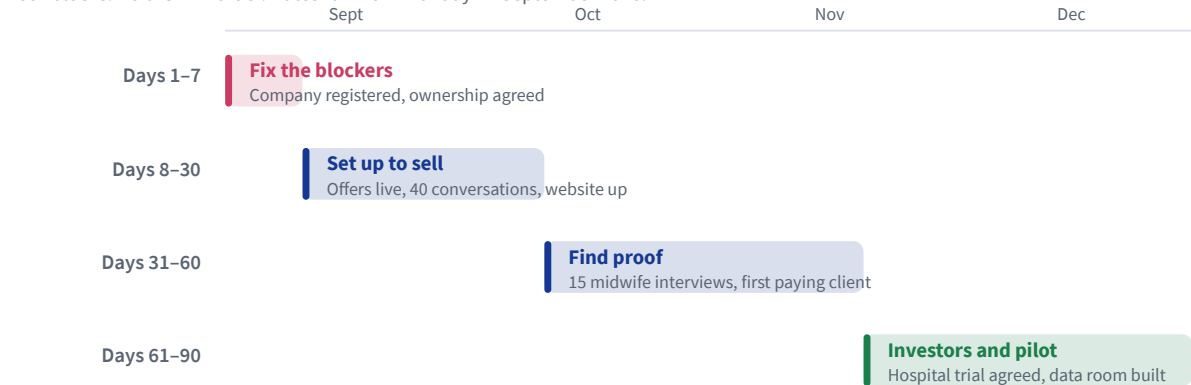
What to do

Action	Who	By when
Open the four blockers: name check, company decision, ownership talk, old claims withdrawn	You	Mon 14 Sept 2026
Get the standstill letter signed and send the first 40 sales messages	You	Fri 18 Sept 2026
Register the company	You and the accountant	Fri 25 Sept 2026
First signed client, and the ownership deed signed	You and the solicitor	Tue 13 Oct 2026
Decide Ekiti, then decide whether to build MaternaLink	You, with the board	Thu 12 Nov and Tue 8 Dec 2026

The ninety days at a glance

The first 90 days

Four blocks. Do them in order. Dates run from Monday 14 September 2026.



Fix the basics first; everything else waits behind those seven days.

Week one, day by day

Day one is Monday 14 September 2026.

Day	What you do	Why	What you end up with
Mon 14 Sept	Set your weekly rhythm and your two trackers. Run the name and trademark checks on "Vytalix" and "MaternaLink" at Companies House, UK IPO, EUIPO and USPTO. Decide to register one English company. Send David Agunede a meeting request with your written questions and the ownership questionnaire. Write down that the 2025 AI deck and the £36m Ekiti headline are withdrawn from all outside use.	Every later step needs a name, a company and a clear owner. Nothing you say in public is safe until the old claims are gone.	Two decisions logged, search results saved with dates, a meeting booked for Wednesday, the old deck withdrawn
Tue 15 Sept	Set up the free CRM (the software that holds your contacts and deals). Build the first 50 of a 150-name list. Rewrite your LinkedIn profile so it says "pre-seed" and "in development". Publish your first post. Ask three solicitors and three accountants to quote.	You cannot sell without a list, and you cannot sign anything without a solicitor.	CRM live, 50 named contacts with sources, an honest profile, six quotes coming
Wed 16 Sept	Hold the ownership conversation with David Agunede, one hour, notes written the same day. Establish who wrote the code and the decks, who pays for the prototype, whether real patient data was ever entered, and what happened after the 18 March 2026 Ekiti email. Hand him the 60-day standstill letter. Adopt the new MaternaLink description: it helps midwives coordinate care, it does not diagnose. Take the old claims out of every file.	Undocumented ownership and over-claiming are the two things that end an investor call in minutes.	A written note of the facts, the letter with him for signature, the new product wording adopted, zero old claims left
Thu 17 Sept	Take the trademark attorney's opinion call on "Vytalix". Choose the solicitor and send the instruction. Send the standstill letter for signature. Send the first 20 sales messages. Publish your third post.	Tomorrow's name decision needs a professional opinion, not your guess. The first client is due on day 30, so selling starts now.	Attorney memo requested, solicitor engaged, 20 messages logged
Fri 18 Sept	Decide the name. If the checks are clean, use it; if not, use the reserve name rather than wait. Buy the domain and set up email. Sign both engagement letters. Get the standstill signed. Send messages 21 to 40. Log how much cash you have. Hold your first Friday review.	A cleared name and a professional bench are what week two is built on.	Name decision, domain and email, two engagement letters, a signed standstill, 40 messages sent, a 13-week cash plan

Weekend: read the draft ownership terms on Saturday, send them to David Agunede on Sunday 20 September.

Weeks two to four

Week	What you do	What you end up with
Week 2, 21 to 27 Sept	File the incorporation (target Wed 23, hard deadline Fri 25). Sign your own IP assignment the same day. Send contract templates to the solicitor. Get insurance quotes. Apply for the bank account. Hold your first two discovery calls and send proposal one. Start the prototype audit.	A registered company, a bank application in, first buyer feedback
Week 3, 28 Sept to 4 Oct	Full sales quota begins: 40 contacts, 20 conversations, 5 discovery calls, 2 proposals each week. Register with the ICO. Bind the insurance. Open the bank account. Start the SEIS and EIS paperwork. Draft the grant calendar and the data room list. Build the 30-name list for UK interviews.	Insured, registered, banked, and selling at full rate
Week 4, 5 to 11 Oct	Adopt the new pricing memo, so the £300 per woman per year figure is gone from every document. Ask for the Ekiti status call and hold it. Approach the first three advisers. Publish the website. Send newsletter one. Negotiate the first engagement.	Website live, honest pricing, Ekiti facts known, data room version one
Days 29 to 30, 12 to 13 Oct	Sign the first client and raise the invoice. Execute the ownership deed. Receive the prototype audit report. Submit the SEIS and EIS application. Hold the first board meeting.	Revenue exists, ownership is documented, phase two is closed

Days 31 to 60

Fortnight	What you do	What you end up with
Days 31 to 45, 14 to 28 Oct	Check that the company, the ownership and the claims are all closed; only then start investor outreach. Contact the top 25 investors, phone first. Decide whether to bid for a grant (by Fri 23 Oct). File the trademark (Wed 28 Oct). Identify a Clinical Safety Officer. Keep the sales quota. Run UK interviews.	25 investors contacted, trademark filed, first meetings booked
Days 46 to 60, 29 Oct to 12 Nov	Contact the next 50 investors. Hold the mDoc conversation and score two other Nigerian states. Decide Ekiti (Thu 12 Nov). Sign two advisers and engage the Clinical Safety Officer. Complete 15 UK interviews and draft the research report. Sign the second client.	75 investors contacted, 4 meetings, Ekiti decided, second client signed

Days 61 to 90

Fortnight	What you do	What you end up with
Days 61 to 75, 13 to 27 Nov	Publish the research report (Fri 20 Nov) and run the webinar (Tue 17 Nov). Start on the next 75 investors. Act on the Ekiti decision by Fri 27 Nov. Keep second meetings moving with 48-hour answers.	Report published, webinar delivered, Nigeria path chosen
Days 76 to 90, 28 Nov to 12 Dec	Sign the third client. Make the MaternaLink build decision (Tue 8 Dec): build, build small, or wait. Get Cyber Essentials certified. Hold the fourth adviser meeting and the third board meeting. Write the plan for the next quarter (Sat 12 Dec).	Three clients, £15k to £30k contracted (ESTIMATE, our best guess), a minuted product decision, a Q1 plan

Four decisions you must make

Decision	Decide by	If yes	If no
Register the company	Decide Mon 14 Sept; filed by Fri 25 Sept	You can invoice, insure, bank, and apply for tax relief and grants	Every later item slips day for day; delay past 25 September is not acceptable
Settle the ownership with David Agunede	Terms sent Sun 20 Sept; deed signed Tue 13 Oct; hard stop Thu 12 Nov	You can say Vytalix owns MaternaLink and name it to investors	Until it is signed, call it "a concept Vytalix is developing with a collaborator". If nothing is signed by 12 November, rebuild version one from scratch and drop his materials and the "Maternal Link" name
Re-scope MaternaLink	Wed 16 Sept	Version one coordinates care and does not diagnose; the AI risk score moves to a research track for later	Keeping the prediction claims needs a medical device route, data and ethics you do not have and cannot fund
Decide on Ekiti	Thu 12 Nov, after the status call, the mDoc conversation and a two-state scorecard	Offer a small, re-priced, partner-aware pilot, and only with board approval	Move to a better-scoring state with an NGO, or stop Nigerian government work for six months. The £4.5m and £300 per woman proposal is not restated under any option

How to decide anything else

Score any idea out of 100. Ten points, each marked 1 to 10.

Point	A high score means
1 Strategic fit	It matches the plan: services pay the bills, MaternaLink is the long story, UK first

Point	A high score means
2 Money	Real cash within 12 to 24 months, and a large repeat stream later
3 Demand	A named buyer has said they want it and can pay
4 Speed	First paying customer in weeks, not years
5 Cost	Cheap to reach the first result, under £2,000
6 Growth	It grows without more of your hours
7 Advantage	A real difference from named competitors that you can build in a year
8 Rules risk	Little exposure to medical device, data or clinical safety rules
9 Simplicity	One owner, known steps, no outside dependency
10 You can do it	You can deliver it alone, with the time and skills you have now

Points 5, 8 and 9 are scored backwards on purpose. Cheap, low risk and simple all score high, so a bigger total is always better.

Total	Verdict	What it means
70 or more	BUILD	Commit your time and money now; it goes in the plan with dates
50 to 69	TEST	Promising but unproven; run one bounded experiment with a budget cap and a decision date
35 to 49	WAIT	Not now; write down the trigger that brings it back
Under 35	STOP	Not pursued as stated; the idea underneath may return in a different form

Four rules beat the total. If strategic fit is 3 or less, it is STOP. If rules risk is 2 or less, WAIT is the best it can be. If you cannot do it alone and cannot pay a team, WAIT. If nobody has said they want it, it can never be BUILD.

Scores for the eight live ideas, made 8 September 2026. All are ESTIMATES (our best guess, not facts).

Idea	Score	Verdict
A. Advisory work you deliver yourself	76	BUILD
B. A research report used to attract clients	65	TEST
C. MaternaLink version one, care coordination only	59	TEST
D. Chasing an NHS pilot, starting as a design partner	54	TEST
E. Launching e-learning in year one	49	WAIT
F. White-label MaternaLink for NGOs	45	WAIT
G. The MaternaLink AI risk score as a product now	33	STOP for now (rules risk)
H. The Ekiti proposal at £300 per woman per year as written	33	STOP as written (no proven buyer)

Re-score every quarter, first in the week of Monday 14 December 2026. Any score that moves by three points or more must name the new evidence.

What not to waste time on

- Any investor conversation before the company, the ownership and the claims are settled. It is the fastest way to lose a good investor.
- Building the MaternaLink app before the decision on Tuesday 8 December. You do not yet know what to build or who pays.
- The AI risk score as a product. It needs a medical device route, a data partner and an ethics review you cannot fund this year.
- Repeating the £300 per woman per year figure or the £36m Ekiti number. Both are withdrawn.
- E-learning courses and white-label versions. Both are WAIT; a waiting list page is all they get.
- Paid logos, printed brand work or a fancy website before the name is cleared on Friday 18 September.
- Hiring anyone. A virtual assistant only if admin passes eight hours a week for two weeks running.

Words explained

Word	What it means
Standstill letter	A short signed promise that neither side sells, licenses or shows MaternaLink to anyone else for 60 days while you agree terms
Heads of terms	A one or two page summary of a deal, agreed before the solicitors write the full contract
Discovery call	A first 30 to 45 minute call with a possible client, to find out what they need
SEIS and EIS	Tax schemes that give people money back when they invest in a small British company
ICO	The UK office you must register with, and pay £52 a year, if you hold people's personal data
Data room	One tidy folder of documents an investor reads before deciding
Cyber Essentials	A basic UK government security certificate that buyers ask for